Documentation Principles and the Failures That Recur

Legibility, timeliness, attribution, correction — and the small habits that destroy a record's credibility.

Module	03 of 12
Study time	100 minutes
Audience	All clinical and nursing staff, medical records staff, quality and risk teams
Assessment	Knowledge check plus a 50-question scored bank
Standards basis	NABH 6th Edition IMS chapter; MOM chapter on abbreviations; professional conduct regulations

The same handful of documentation failures appear in every hospital audit, in every accreditation report, and in almost every case that is lost. They are not knowledge failures — every clinician knows that entries should be signed and timed. They are discipline failures, and they cluster at predictable points: night shifts, busy lists, handovers, and the moment an error is discovered. This module covers the principles, the correct method for every situation where staff improvise, and the abbreviations that must never be used.

LEARNING OUTCOMES — by the end of this module you will be able to:

- Apply the ten principles of sound documentation to your own entries
- Correct an error in the record by the only acceptable method
- Distinguish a late entry, an addendum, an amendment and an entry made in error, and write each correctly
- Explain why obliteration damages a hospital more than the original error
- Use the hospital's approved abbreviation list and avoid every item on the prohibited list
- Document verbal instruction, countersignature and supervision of trainees correctly
- Recognise the circumstances in which documentation predictably fails and build defences against them

3.1 The ten principles

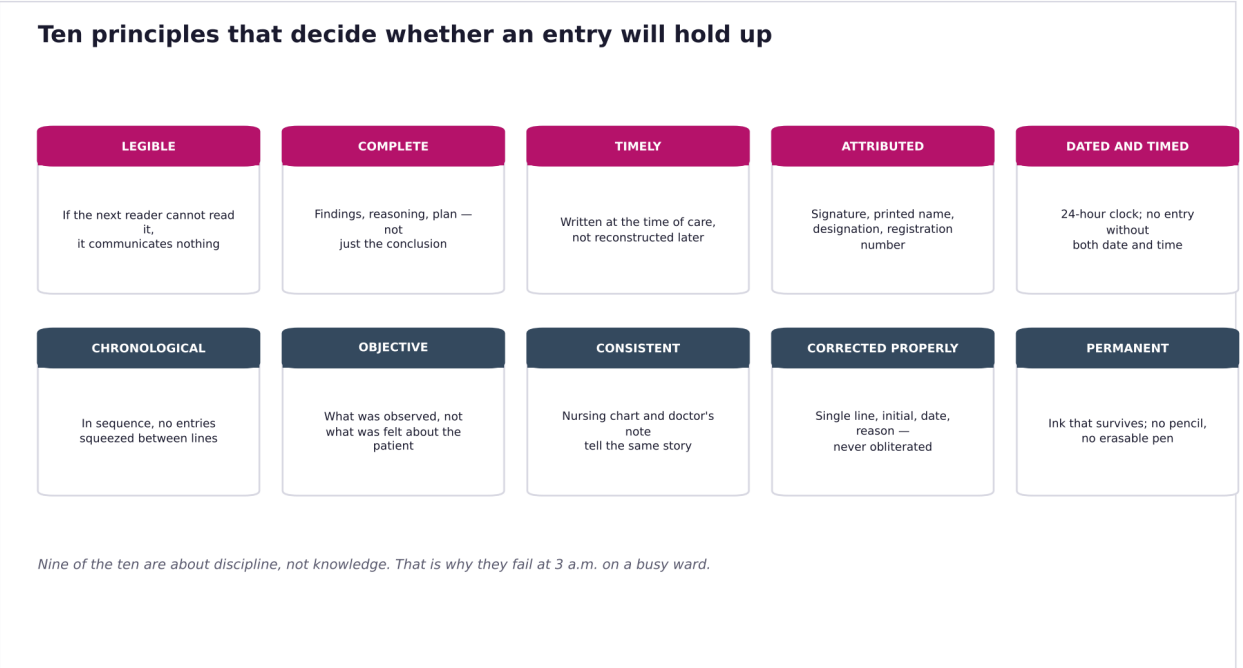


Figure 3.1 — Ten principles that determine whether an entry will stand up.

Each principle exists because of a specific failure mode. Legibility exists because illegible prescriptions kill people. Attribution exists because an unattributable entry cannot be questioned or defended. Chronology exists because disputes about care are disputes about sequence. Objectivity exists because a judgmental phrase written in frustration becomes the sentence read aloud in the hearing.

Objectivity in practice

Objectivity is the principle staff find hardest, because clinical work generates frustration and the record is the nearest outlet. The rule is to record behaviour, not character; observation, not inference about motive; and the patient's own words in quotation marks where they matter.

Avoid	Write instead
"Patient is uncooperative and difficult."	"Patient declined the morning dose of metformin, saying it 'makes me feel sick'. Explained the reason for the medication; patient still declined. Consultant informed at 09:40."
"Attender is being aggressive."	"Patient's brother raised his voice at the nursing station at 22:15 and demanded an immediate consultant review. Duty officer called; situation settled by 22:30. No physical contact."
"Patient appears to be drug-seeking."	"Patient has requested additional analgesia four times in six hours. Pain score 8/10 on each occasion. Last dose given at 14:00. Discussed with the treating team; plan reviewed."
"Obviously non-compliant with treatment."	"Patient reports taking insulin on about three days of the past week, citing cost. Referred to the diabetes educator and to medical social work."
"The night staff did not monitor the patient properly."	"No vital signs recorded between 00:00 and 06:00. Incident reported under the hospital incident reporting system, reference number noted."

CAUTION
Never use the record to criticise a colleague, allocate blame, or comment on another department. Record the facts and raise the concern through the incident reporting system. Blame written into a clinical record is admissible, quotable, and converts a defensible case into an internal contradiction the hospital has to explain.

3.2 Legibility and the cost of handwriting

Illegible records cause three distinct harms: a drug is misread, a finding is lost, and in proceedings the hospital cannot rely on the entry because nobody can agree what it says. The practical standard is not beautiful handwriting but the ability of another reader, unfamiliar with the author, to read every word.

- Write in permanent dark ink. Pencil and erasable pens have no place in a medical record
- Print rather than join letters where handwriting is poor, especially for drug names and numbers
- Write drug names in full; abbreviated and branded names are the main source of misreading
- Avoid writing in the margin, between lines, or vertically up the side of the page
- If an entry cannot be read back by a colleague, rewrite it as a dated addendum rather than overwriting it
- Where a hospital has moved to electronic prescribing, the legibility problem transfers to selection errors from drop-down lists, which needs its own check

3.3 Correcting an error

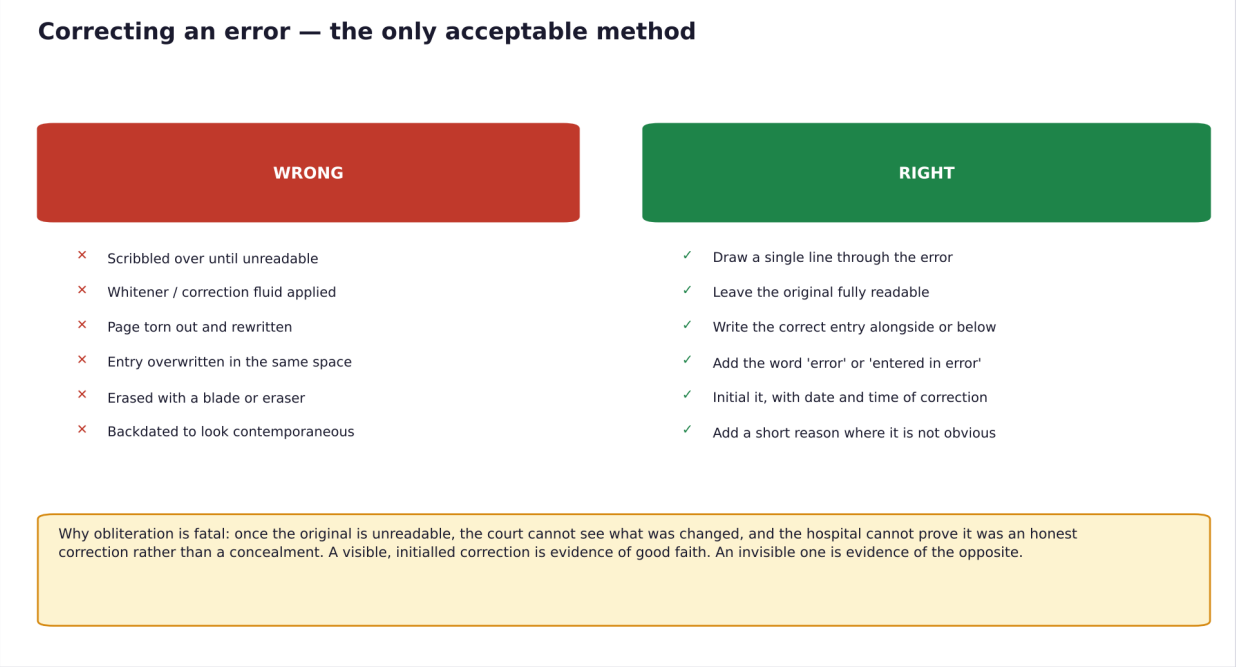


Figure 3.2 — The only acceptable method of correcting an entry.

Everybody makes errors in records. The hospital is not damaged by the error; it is damaged by the method of correction. A single line through the error, leaving it readable, with the correct entry alongside and the correction initialled, dated and timed, is evidence of an honest process. Whitener, scribbling, overwriting, removing a page or writing a fresh page to replace it are each read as concealment, and they taint the whole record, not just the entry.

In an electronic record the same principle applies in a different form. The original entry must be preserved and visible as a prior version; the correction must be a new entry with its own author and timestamp; and the system must be able to show both. “Editing” an entry so that only the new text survives is the electronic equivalent of whitener.

> **KEY POINT**
> A hospital that can show a visible, initialled correction is showing a system that catches and fixes errors. A hospital that produces a file with an obliterated entry is showing a system that hides them. The difference in how those two files are received is very large.

3.4 Late entries, addenda and amendments

Late entries, addenda and amendments — which is which

Situation	What it is called	How it is written	Risk if done wrongly
Care given, note not written in the shift	Late entry	Date and time of writing, then 'late entry for [date and time of care]'	Reads as fabrication if the writing time is hidden
Note written, new information arrives later	Addendum	A new dated and timed entry referring to the original; the original is left untouched	Editing the original destroys the chronology
Note contains a factual error	Amendment / correction	Single line through, correct entry alongside, initial, date, time, reason	Obliteration reads as concealment
Entry made in the wrong patient's file	Entered in error	Struck through, marked 'entered in error' and signed; the correct file updated separately	Deleting it leaves two records disagreeing

Never write a late entry as though it were contemporaneous. The hospital's defence depends on the record being honest about itself.

Figure 3.3 — Which situation calls for which kind of entry.

Staff improvise here more than anywhere else, usually with good intentions. A nurse who realises at the end of a shift that an intervention was not charted writes it in among the earlier entries so the chart “looks right”. That is the single most damaging thing she can do, because it converts an ordinary omission into an entry that is demonstrably not contemporaneous.

Writing a late entry correctly

1. Start the entry at the current point in the notes, in sequence — never squeezed into an earlier space.
2. Date and time the entry with the date and time you are writing it.
3. Label it explicitly: “Late entry for 14/03 at approximately 22:30”.
4. Record the care or observation as it occurred, and say if any element is approximate.
5. If relevant, state briefly why the entry is late — for example that the ward was responding to a cardiac arrest.
6. Sign in full with name, designation and registration number.

A late entry written this way is unremarkable and is accepted. The same information written to look contemporaneous is a gift to the other side, because ink colour, page position, handwriting pressure and electronic audit trails all reveal it, and once one entry is shown to be false the whole file is treated with suspicion.

3.5 Abbreviations

Two separate documents are required: a list of abbreviations the hospital approves for use, and a list of abbreviations, symbols and dose designations that must never be used. Both must be current, both must be accessible at the point of care, and the prohibited list must be audited.

Do-not-use abbreviations — the core list

Written	Read as	Intended	Write instead
U or u	0, 4 or cc	unit	unit
IU	IV or 10	international unit	international unit
Q.D. / QD / qd	QID	once daily	daily
Q.O.D. / QOD	QD or QID	every other day	every other day
Trailing zero 1.0 mg	10 mg	1 mg	1 mg
No leading zero .5 mg	5 mg	0.5 mg	0.5 mg
MS / MSO4 / MgSO4	confused with each other	morphine or magnesium	write the drug name in full
µg	mg	microgram	mcg or microgram
cc	u (units)	millilitre	mL
AD / AS / AU · OD / OS / OU	each other	ear or eye, side	right ear, left eye, both eyes
D/C	discharge or discontinue	either	write the word intended
@	2	at	at

The hospital-approved abbreviation list and this prohibited list are two separate documents. Both must exist and both must be current.

Figure 3.4 — The core do-not-use list.

The items on the prohibited list are not there because they are untidy. Each one has a documented history of harm. “U” for unit is read as a zero, turning 4U of insulin into 40. A trailing zero turns 1.0 mg into 10 mg. A missing leading zero turns 0.5 mg into 5 mg. “MS” has been read as both morphine sulphate and magnesium sulphate in the same hospital. These are ten-fold errors in drugs with narrow margins.

- Write the word “unit” in full, every time, in every setting
- Never place a zero after a decimal point; always place one before it
- Write drug names in full, in generic form, including for infusions and chemotherapy
- Write the intended word where an abbreviation is ambiguous: “discharge” or “discontinue”, not D/C
- Spell out route and site where confusion is possible: “right eye”, not OD
- Apply the prohibited list to electronic prescribing too, including free-text fields and order comments

> **CAUTION**
> The prohibited list applies to every document in the record, not only to prescriptions. A progress note that records “patient given 10u insulin” carries the same ambiguity as a chart entry and will be read by the same audit.

3.6 Verbal instructions, countersignature and supervision

Verbal instructions are covered in full in Module 5 for medication. The documentation principle is broader: whenever a clinical instruction is given by someone who is not writing it down, the person who writes it must record who gave it, when, and that it was read back and confirmed. The person who gave it must authenticate it afterwards within the hospital’s stated time.

Countersignature is often misunderstood as a formality. A countersignature asserts that the supervising clinician takes responsibility for the entry — which means it should not be given in a batch, days later, without reading what is being signed. In proceedings, a countersignature is treated as the supervisor’s own adoption of the content.

Situation	Who writes	Who authenticates and when
Verbal or telephone medication order	The nurse receiving it, after read-back	The prescriber, within twenty-four hours
Entry by an intern or resident in training	The trainee, in full, with designation	The supervising clinician as required by hospital policy, having read the entry
Entry by a nursing student	The student, identified as a student	The registered nurse supervising, who remains accountable for the care
Transcription of an order from one chart to another	The person transcribing, identified	Checked against the original by a second person where policy requires
Telephonic report of a critical result	Both the laboratory and the receiver, each in their own record	No countersignature substitutes for the clinician's own note of the action taken

3.7 Blank spaces, unused fields and the pre-signed form

Three related habits each create risk. A blank space left in a running entry allows an addition later, so entries are closed by drawing a line through unused space on the line. A form with fields left empty is ambiguous: the reader cannot tell whether the finding was normal, was not assessed, or was assessed and not recorded, so "not assessed" or "nil" should be written rather than nothing. And a form signed before it is completed — a consent form signed in advance of the explanation, a checklist signed before the checks — is a false record, regardless of whether the steps later happened.

> **CAUTION**
> Pre-signing a checklist is one of the few documentation failures that is treated as deliberate falsification rather than carelessness. It is also easy to prove, because the signature exists before the events it attests to.

3.8 Where documentation predictably fails

Documentation does not fail randomly. It fails in the same circumstances in every hospital, which means the defences can be built in advance.

Circumstance	Why it fails	What defends against it
Night shift and weekends	Fewer staff, higher acuity, no clerical support, and nobody auditing in real time	A short, mandatory night checklist; the morning handover explicitly checking that overnight events are charted
Emergency and resuscitation	Everybody is treating; nobody is writing; times are reconstructed afterwards	A designated scribe role in the arrest team, with a timed resuscitation form; the record completed and signed before the team disperses
Shift change and handover	The outgoing staff member intends to chart after handover and then leaves	Charting before handover as a rule; the handover record itself retained
Busy operating lists	Operative notes deferred to the end of the list and then written from memory for several patients	A brief operative note written in theatre for each case, with the full note the same day
Transfer between units	Each unit assumes the other documented the handover	A transfer form that must be completed by the sending unit and signed on receipt by the receiving unit
Consultant ward rounds	Decisions are made verbally and the junior writes a summary later, losing the reasoning	The decision and its reason recorded during the round; the consultant reviewing and authenticating the entry

Circumstance	Why it fails	What defends against it
Discharge on a Friday evening	Summary written in haste or deferred to Monday	Discharge summary completion as a condition of discharge, not an administrative follow-up

3.9 Self-check before you close an entry

1. Is the patient identified on this page?
2. Is the date there, and the time on the twenty-four hour clock?
3. Can another person read every word, including every drug name and number?
4. Have I recorded findings and reasoning, or only a conclusion?
5. Does this entry say what I will do next, and what would make me act sooner?
6. Have I signed it, printed my name, and given my designation and registration number?
7. Have I avoided every abbreviation on the prohibited list?
8. If I corrected anything, is the original still readable, with my initials, the date and the time?
9. Would I be comfortable if this entry were read aloud, in full, in a hearing?

3.10 Language, interpreters and the record

India's hospitals routinely treat patients who do not share a language with the clinician. The documentation consequences are specific and they bear on consent, on history-taking and on discharge advice.

- Record which language the consultation was conducted in
- Where an interpreter was used, name them and record their relationship to the patient; a family member interpreting is common and should be recorded as such, because it affects reliability and privacy
- Never use a child as an interpreter for a clinical conversation, and if it happens in an emergency, record why
- Where the patient cannot read the language of the consent form, record who read and explained it and in which language
- Write the discharge instructions in a language the patient can act on, or record that they were explained and to whom
- Keep the clinical record itself in the hospital's documented language of record, with the patient's own words quoted in their language where the exact words matter

For a patient who cannot sign, a thumb impression is taken with an independent witness who is not a member of the treating team, and the record states who explained the content and in what language. A thumb impression with no witness and no record of explanation is routinely challenged and is difficult to defend.

3.11 Dates, times and numbers

A surprising proportion of documentation disputes turn on ambiguity in how a date or a number was written.

Problem	Why it matters	Rule
Ambiguous date formats	03/04 is read as 3 April or as 4 March depending on the reader	Use an unambiguous format throughout the hospital; writing the month in letters removes the ambiguity entirely
Twelve-hour clock without am or pm	'8.30' could be twelve hours either side of the event	Twenty-four hour clock everywhere, with four digits
Entries crossing midnight	A night-shift entry dated to the previous day, or to the next, inconsistently	The date changes at 00:00; the shift does not determine the date; record both date and time on every entry

Problem	Why it matters	Rule
Decimal points in doses	A misread decimal is a ten-fold error	Leading zero always; trailing zero never; avoid decimals where a whole number in smaller units will do
Ranges and approximations	‘Pain 6-7’ cannot be trended	A single value from the tool in use
Units omitted	A number with no unit is not a measurement	Every value carries its unit, written out where the abbreviation is on the prohibited list

3.12 Students, non-registered staff and who may write

Hospitals must define who may make entries in the medical record. The definition should be written, and it should cover students, interns, non-registered technical staff, scribes and contracted staff.

Who	May write	Conditions
Registered doctors and nurses	Yes, within their scope	Full authentication with registration number
Interns and residents	Yes	Identified by designation; countersigned as hospital policy requires
Nursing and medical students	Yes, identified as students	Entries countersigned by the supervising registered professional, who remains accountable
Allied health professionals	Yes, within their scope	In their own section of the record, authenticated
Technicians and operators	Procedure and device records only	Within their defined scope, authenticated
Clerical staff and scribes	Administrative data only, or dictated clinical content where the hospital has a scribe policy	Clinical content attributed to and authenticated by the clinician who dictated it, after review
Contracted or outsourced staff	Only where the contract and the hospital policy permit	Confidentiality undertaking signed; access controlled and logged

3.13 Writing when you disagree

Clinicians disagree, and the record is not the place to argue. But a clinician who believes a decision is wrong has a professional obligation to record their own position, and there is a way to do that which protects the patient without damaging the hospital.

1. Record your own findings and your own clinical opinion, in full, as an assessment.
2. Record what you recommended and to whom you communicated it, with the time.
3. Record the decision that was taken and by whom, factually and without comment.
4. Record what you did next — further monitoring, a second opinion sought, escalation to a senior.
5. Do not characterise the other clinician’s reasoning, competence or motive.
6. If the concern is serious, raise it through the incident reporting or clinical governance route, and record that you have done so.

Written this way, the record shows a clinician who acted responsibly. Written as criticism, the same facts produce an internal contradiction that the hospital must explain and that helps nobody, least of all the patient.

3.14 The physical and electronic security of what you write

- Do not leave files open at a nursing station visible to passing relatives
- Do not leave a workstation logged in and unattended; use the lock function rather than relying on a timeout

- Position screens so that they are not readable from a waiting or public area
- Do not discuss a patient in a corridor, lift, canteen or any space where you cannot see who is listening
- Do not photograph records, and do not send clinical information through personal messaging accounts
- Report a lost document or a suspected unauthorised access immediately, on the day it is noticed

Knowledge Check

Work through each item before reading the model answer. These are applied questions: the answer is the standard of documentation your assessor, your medico-legal adviser or the next clinician to read the file would expect.

Q1. **A nurse realises at 06:00 that she gave an antibiotic at 02:00 and did not sign for it. She signs the 02:00 box now. What is wrong, and what should she do instead?**

Answer: Signing the 02:00 box makes a false contemporaneous entry: it asserts that the signature was made at the time of administration. She should sign and date the administration record at the current time, annotate it as a late entry for 02:00, and write a short nursing note recording the drug, dose, route, the actual time of administration and the reason for the late entry. Done this way it is an ordinary omission honestly recorded; done the other way it is a falsified record.

Q2. **A doctor writes a progress note, realises it was entered in the wrong patient's file, and tears the page out. What are the consequences and what was the correct action?**

Answer: Removing a page destroys the integrity of the file: the page numbering breaks, the chronology has a gap, and if it is ever noticed the hospital has no innocent explanation. The correct action is to strike through the entry with a single line, leave it readable, write 'entered in error — see correct file', sign, date and time it, and then make the entry in the correct patient's file as a late entry. Both files are then internally consistent and the error is visible and explained.

Q3. **A prescription reads 'Inj MS 4U stat'. List every documentation defect in those five characters and state what should have been written.**

Answer: 'MS' is prohibited and ambiguous between morphine sulphate and magnesium sulphate. 'U' is prohibited and reads as a zero, so 4U can be read as 40. There is no route, no concentration, no indication, and 'stat' without a time does not establish when it was due. It should read, for example: 'Inj Morphine Sulphate 4 mg intravenous, stat at 14:20, for acute pain', written in full with the prescriber's signature, name, registration number, date and time.

Q4. **An anaesthetist countersigns a week of trainee entries in one sitting without reading them. One entry contains a serious factual error. What is the anaesthetist's position?**

Answer: A countersignature is treated as adoption of the content. By signing, the anaesthetist has asserted supervision of entries they did not read, so they carry responsibility for the error and additionally for a supervision process that plainly did not operate. Countersignature must be contemporaneous enough to be meaningful, and must involve actually reading the entry; batch signing is itself a finding.

Q5. **A ward uses a printed nursing assessment form on which several fields are routinely left blank when the finding is normal. Why is this a problem?**

Answer: A blank field is ambiguous: a later reader cannot tell whether the item was normal, was not assessed, or was assessed and not recorded. In a dispute, the hospital cannot show that a pressure-injury or falls risk assessment was carried out at all. Every field should be completed, with 'nil', 'normal' or 'not assessed' written as appropriate, and unused space on an entry line should be ruled through.

Q6. **A consultant writes in the notes: 'Previous unit mismanaged this patient; antibiotics should have been started two days ago.' What is the problem and what should have been recorded?**

Answer: It is an opinion about another clinician's care recorded in a clinical document that will be disclosed in full. It creates an admission against the hospital, it is often based on incomplete information, and it does nothing for the patient's care. The record should state the clinical facts — the current findings, the time antibiotics were started by this team, and the plan. Any concern about the earlier care goes through the incident reporting or clinical governance route, not into the patient's notes.

> **ASSESSMENT BANK**
> A fuller bank of 50 scored questions for this module is supplied separately as a JSON question bank for loading into your LMS.